

Histrionic personality disorder

Histrionic personality disorder (HPD) is defined by the American Psychiatric Association as a personality disorder characterized by a pattern of excessive attention-seeking behaviors, usually beginning in early childhood, including inappropriate seduction and an excessive desire for approval. People diagnosed with the disorder are said to be lively, dramatic, vivacious, enthusiastic, extroverted and flirtatious.

Histrionic personality disorder	
Specialty	<u>Clinical Psychology</u> , <u>Psychiatry</u>
Symptoms	Persistent <u>attention seeking</u> , <u>exhibitionism</u>

HPD lies in the dramatic cluster of personality disorders.^[1] People with HPD have a high desire for attention, make loud and inappropriate appearances, exaggerate their behaviors and emotions, and crave stimulation.^[1] They may exhibit sexually provocative behavior, express strong emotions with an impressionistic style, and can be easily influenced by others. Associated features include egocentrism, self-indulgence, continuous longing for appreciation, and persistent manipulative behavior to achieve their own wants.

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Signs and symptoms

People with HPD are usually high-functioning, both socially and professionally. They usually have good social skills, despite tending to use them to manipulate others into making them the center of attention.^[2] HPD may also affect a person's social and romantic relationships, as well as their ability to cope with losses or failures. They may seek treatment for clinical depression when romantic (or other close personal) relationships end.^[3]

Individuals with HPD often fail to see their own personal situation realistically, instead dramatizing and exaggerating their difficulties. They may go through frequent job changes, as they become easily bored and may prefer withdrawing from frustration (instead of facing it). Because they tend to crave novelty and excitement, they may place themselves in risky situations. All of these factors may lead to greater risk of developing clinical depression.^[4] Research has also shown those with histrionic personality belief to have a greater desire for social approval, making those with HPD more vulnerable to social media addiction.^[5]

Additional characteristics may include:

- Exhibitionist behavior
- Constant seeking of reassurance or approval
- Excessive sensitivity to criticism or disapproval
- Pride of own personality and unwillingness to change, viewing any change as a threat
- Inappropriately seductive appearance or behavior of a sexual nature
- Using factitious somatic symptoms (of physical illness) or psychological disorders to garner attention
- Craving attention
- Low tolerance for frustration or delayed gratification
- Rapidly shifting emotional states that may appear superficial or exaggerated to others
- Tendency to believe that relationships are more intimate than they actually are
- Making rash decisions^[2]
- Blaming personal failures or disappointments on others
- Being easily influenced by others, especially those who treat them approvingly
- Being overly dramatic and emotional^[4]
- Influenced by the suggestions of others^[6]

Some people with histrionic traits or personality disorder change their seduction technique into a more maternal or paternal style as they age.^[7]

Causes

Little research has been done to find evidence of what causes histrionic personality disorder. Although direct causes are inconclusive, various theories and studies suggest multiple possible causes, of a neurochemical, genetic, psychoanalytic, or environmental nature. Traits such as extravagance, vanity, and

seductiveness of hysteria have similar qualities to women diagnosed with HPD.^[8] HPD symptoms typically do not fully develop until the age of 15, while the onset of treatment only occurs, on average, at approximately 40 years of age.^{[9][10]}

Neurochemical/physiological

Studies have shown that there is a strong correlation between the function of neurotransmitters and the Cluster B personality disorders such as HPD. Individuals diagnosed with HPD have highly responsive noradrenergic systems which is responsible for the synthesis, storage, and release of the neurotransmitter, norepinephrine. High levels of norepinephrine leads to anxiety-proneness, dependency, and high sociability.^[9]

Genetic

Twin studies have aided in breaking down the genetic vs. environment debate. A twin study conducted by the Department of Psychology at Oslo University attempted to establish a correlation between genetic and Cluster B personality disorders. With a test sample of 221 twins, 92 monozygotic and 129 dizygotic, researchers interviewed the subjects using the Structured Clinical Interview for DSM-III-R Personality Disorders (SCID-II) and concluded that there was a correlation of 0.67 that histrionic personality disorder is hereditary.^[11]

Psychoanalytic theory

Though criticised as being unsupported by scientific evidence, psychoanalytic theories incriminate authoritarian or distant attitudes by one (mainly the mother) or both parents, along with conditional love based on expectations the child can never fully meet.^[1] Using psychoanalysis, Freud believed that lustfulness was a projection of the patient's lack of ability to love unconditionally and develop cognitively to maturity, and that such patients were overall emotionally shallow.^[12] He believed the reason for being unable to love could have resulted from a traumatic experience, such as the death of a close relative during childhood or divorce of one's parents, which gave the wrong impression of committed relationships. Exposure to one or multiple traumatic occurrences of a close friend or family member's leaving (via abandonment or mortality) would make the person unable to form true and affectionate attachments towards other people.^[13]

HPD and antisocial personality disorder

Another theory suggests a possible relationship between histrionic personality disorder and antisocial personality disorder. Research has found 2/3 of patients diagnosed with histrionic personality disorder also meet criteria similar to those of the antisocial personality disorder,^[8] which suggests both disorders based towards sex-type expressions may have the same underlying cause.

Some family history studies have found that histrionic personality disorder, as well as borderline and antisocial personality disorders, tend to run in families, but it is unclear how much is due to genetic versus environmental factors.^[14] Both examples suggest that predisposition could be a factor as to why certain people are diagnosed with histrionic personality disorder, however little is known about whether or not the disorder is influenced by any biological compound or is genetically inheritable.^[14] Little research has been conducted to determine the biological sources, if any, of this disorder.

Diagnosis

The person's appearance, behavior and history, along with a psychological evaluation, are usually sufficient to establish a diagnosis. There is no test to confirm this diagnosis. Because the criteria are subjective, some people may be wrongly diagnosed.^[15]

DSM 5

The current edition of the Diagnostic and Statistical Manual of Mental Disorders, DSM 5, defines histrionic personality disorder (in Cluster B) as:^[16]

A pervasive pattern of excessive emotionality and attention-seeking, beginning by early adulthood and present in a variety of contexts, as indicated by five (or more) of the following:

- is uncomfortable in situations in which he or she is not the center of attention
- interaction with others is often characterized by inappropriate sexually seductive or provocative behavior
- displays rapidly shifting and shallow expression of emotions
- consistently uses physical appearance to draw attention to self
- has a style of speech that is excessively impressionistic and lacking in detail
- shows self-dramatization, theatricality, and exaggerated expression of emotion
- is suggestible, i.e., easily influenced by others or circumstances
- considers relationships to be more intimate than they actually are

The DSM 5 requires that a diagnosis for any specific personality disorder also satisfies a set of general personality disorder criteria.

ICD-10

The World Health Organization's ICD-10 lists histrionic personality disorder as:^[17]

A personality disorder characterized by:

- shallow and labile affectivity,
- self-dramatization,
- theatricality,
- exaggerated expression of emotions,
- suggestibility,
- egocentricity,
- self-indulgence,
- lack of consideration for others,
- easily hurt feelings, and
- continuous seeking for appreciation, excitement and attention.

It is a requirement of ICD-10 that a diagnosis of any specific personality disorder also satisfies a set of general personality disorder criteria.

Comorbidity

Most histrionics also have other mental disorders. Comorbid conditions include: antisocial, dependent, borderline, and narcissistic personality disorders,^[18] as well as depression, anxiety disorders, panic disorder, somatoform disorders, anorexia nervosa, substance use disorder^[19] and attachment disorders, including reactive attachment disorder.

Millon's subtypes

Theodore Millon identified six subtypes of histrionic personality disorder. Any individual histrionic may exhibit one or more of the following:^[20]

Subtype	Description	Personality Traits
Appeasing histrionic	Including <u>dependent</u> and <u>compulsive</u> features	Seeks to placate, mend, patch up, smooth over troubles; knack for settling differences, moderating tempers by yielding, compromising, conceding; sacrifices self for commendation; fruitlessly placates the unplacatable.
Vivacious histrionic	The seductiveness of the histrionic mixed with the energy typical of <u>hypomania</u> . Some <u>narcissistic</u> features can also be present	Vigorous, charming, bubbly, brisk, spirited, flippant, impulsive; seeks momentary cheerfulness and playful adventures; animated, energetic, ebullient.
Tempestuous histrionic	Including <u>negativistic</u> features	Impulsive, out of control; moody complaints, sulking; precipitous emotion, stormy, impassioned, easily wrought-up, periodically inflamed, turbulent.
Disingenuous histrionic	Including <u>antisocial</u> features	Underhanded, double-dealing, scheming, contriving, plotting, crafty, false-hearted; egocentric, insincere, deceitful, calculating, guileful.
Theatrical histrionic	Variant of "pure" pattern	Affected, mannered, put-on; postures are striking, eye-catching, graphic; markets self-appearance; is synthesized, stagy; simulates desirable/dramatic poses.
Infantile histrionic	Including <u>borderline</u> features	Labile, high-strung, volatile emotions; childlike hysteria and nascent pouting; demanding, overwrought; fastens and clutches to another; is excessively attached, hangs on, stays fused to and clinging.

Treatment

Treatment is often prompted by depression associated with dissolved romantic relationships. Medication does little to affect the personality disorder, but may be helpful with symptoms such as depression.^[15] Treatment for HPD itself involves psychotherapy, including cognitive therapy.^[1]

Interviews and self-report methods

In general clinical practice with assessment of personality disorders, one form of interview is the most popular: an unstructured interview.^[21] The actual preferred method is a semi-structured interview but there is reluctance to use this type of interview because they can seem impractical or superficial.^[21] The reason that a semi-structured interview is preferred over an unstructured interview is that semi-structured

interviews tend to be more objective, systematic, replicable, and comprehensive.^[21] Unstructured interviews, despite their popularity, tend to have problems with unreliability and are susceptible to errors leading to false assumptions of the patient.^[21]

One of the single most successful methods for assessing personality disorders by researchers of normal personality functioning is the self-report inventory following up with a semi-structured interview.^[21] There are some disadvantages with the self-report inventory method that with histrionic personality disorder there is a distortion in character, self-presentation, and self-image.^[21] This cannot be assessed simply by asking most clients if they match the criteria for the disorder.^[21] Most projective testing depend less on the ability or willingness of the person to provide an accurate description of the self, but there is currently limited empirical evidence on projective testing to assess histrionic personality disorder.^[21]

Functional analytic psychotherapy

Another way to treat histrionic personality disorder after identification is through functional analytic psychotherapy.^[22] The job of a Functional Analytic Psychotherapist is to identify the interpersonal problems with the patient as they happen in session or out of session.^[22] Initial goals of functional analytic psychotherapy are set by the therapist and include behaviors that fit the client's needs for improvement.^[22] Functional analytic psychotherapy differs from the traditional psychotherapy due to the fact that the therapist directly addresses the patterns of behavior as they occur in-session.^[22]

The in-session behaviors of the patient or client are considered to be examples of their patterns of poor interpersonal communication and to adjust their neurotic defenses.^[22] To do this, the therapist must act on the client's behavior as it happens in real time and give feedback on how the client's behavior is affecting their relationship during therapy.^[22] The therapist also helps the client with histrionic personality disorder by denoting behaviors that happen outside of treatment; these behaviors are termed "Outside Problems" and "Outside Improvements".^[22] This allows the therapist to assist in problems and improvements outside of session and to verbally support the client and condition optimal patterns of behavior".^[22] This then can reflect on how they are advancing in-session and outside of session by generalizing their behaviors over time for changes or improvement".^[22]

Coding client and therapist behaviors

In these sessions there is a certain set of dialogue or script that can be forced by the therapist for the client to give insight on their behaviors and reasoning".^[22] Here is an example from"^[22] the conversation is hypothetical. T = therapist C = Client This coded dialogue can be transcribed as:

- **ECRB** – Evoking clinically relevant behavior
 - T: Tell me how you feel coming in here today (**CRB2**) C: Well, to be honest, I was nervous. Sometimes I feel worried about how things will go, but I am really glad I am here.
- **CRB1** – In-session problems
 - C: Whatever, you always say that. (becomes quiet). I don't know what I am doing talking so much.
- **CRB2** – In-session improvements
- **TCRB1** – Clinically relevant response to client problems
 - T: Now you seem to be withdrawing from me. That makes it hard for me to give you what you might need from me right now. What do you think you want from me as we are

talking right now?".

- **TCRB2** – Responses to client improvement
 - T: That's great. I am glad you're here, too. I look forward to talking to you.^[22]

Functional ideographic assessment template

Another example of treatment besides coding is functional ideographic assessment template.^[22] The functional ideographic assessment template, also known as FIAT, was used as a way to generalize the clinical processes of functional analytic psychotherapy.^[22] The template was made by a combined effort of therapists and can be used to represent the behaviors that are a focus for this treatment.^[22] Using the FIAT therapists can create a common language to get stable and accurate communication results through functional analytic psychotherapy at the ease of the client; as well as the therapist.^[22]

Epidemiology

The survey data from the National epidemiological survey from 2001 to 2002 suggests a prevalence of HPD of 1.84 percent.^[23] Major character traits may be inherited, while other traits may be due to a combination of genetics and environment, including childhood experiences.^[7] This personality is seen more often in women than in men.^[24] Approximately 65% of HPD diagnoses are women while 35% are men. In Marcie Kaplan's *A Women's View of DSM-III*, she argues that women are overdiagnosed due to potential biases and expresses that even healthy women are often automatically diagnosed with HPD.^[25] It has also been argued due to diagnostic bias that prevalence rates are equal among women and men.^[26]

Many symptoms representing HPD in the DSM are exaggerations of traditional feminine behaviors. In a peer and self-review study, it showed that femininity was correlated with histrionic, dependent and narcissistic personality disorders.^[27] Although two thirds of HPD diagnoses are female, there have been a few exceptions.^[28] Whether or not the rate will be significantly higher than the rate of women within a particular clinical setting depends upon many factors that are mostly independent of the differential sex prevalence for HPD.^[29] Those with HPD are more likely to look for multiple people for attention, which leads to marital problems due to jealousy and lack of trust from the other party. This makes them more likely to become divorced or separated once married.^[30] With few studies done to find direct causations between HPD and culture, cultural and social aspects play a role in inhibiting and exhibiting HPD behaviors.

History

The term 'Histrionic' stems from Etruscan histrion which means "an actor". The Romans adapted the Greek word hystera to 'Histrione', relating to actors who represented 'coarse farces representing those who are false and theatrical.'^[31] Hysteria can be described as an exaggerated or uncontrollable emotion that people, especially in groups, experience. Beliefs about hysteria have varied throughout time. It was not until Sigmund Freud who studied histrionic personality disorder in a psychological manner.^[32] "The roots of histrionic personality can be traced to cases of hysterical neurosis described by Freud."^[12] He developed the psychoanalytic theory in the late 19th century and the results from his development led to split concepts of hysteria. One concept labeled as hysterical neurosis (also known as conversion disorder)^[33] and the other concept labeled as hysterical character (currently known as histrionic personality disorder).^[32] These two concepts must not be confused with each other, as they are two separate and different ideas.^[12]

Histrionic personality disorder is also known as hysterical personality. Hysterical personality has evolved in the past 400 years^[34] and it first appeared in the DSM II (Diagnostic and Statistical Manual of Mental Disorders, 2nd edition) under the name hysterical personality disorder. The name we know today as histrionic personality disorder is due to the name change in DSM III, third edition. Renaming hysterical personality to histrionic personality disorder is believed to be because of possible negative connotations to the roots of hysteria, such as intense sexual expressions, demon possessions, etc.^[35]

Histrionic personality disorder has gone through many changes. From hysteria, to hysterical character, to hysterical personality disorder, to what it is listed as in the most current DSM, DSM-5. "Hysteria is one of the oldest documented medical disorders."^[32] Hysteria dates back to both ancient Greek and Egyptian writings.^[32] Most of the writings related hysteria and women together, similar to today where the epidemiology of histrionic personality disorder is generally more prevalent in women and also frequently diagnosed in women.^[21]

Ancient times

- Ancient Egypt – first description of the mental disorder, hysteria, dates back to 1800 BCE in Ancient Egypt. Biological issues, such as the uterus movement in the female body, were seen as the cause of hysteria. Traditional symptoms and descriptions of hysteria can be found in the Kahun Gynaecological Papyrus, the oldest known medical text in Egypt.^[36]
- Ancient Greece – Similar to ancient Egyptians, the ancient Greeks saw hysteria being related to the uterus. Hippocrates (5th century BCE) is the first to use the term hysteria. Hippocrates believed hysteria was a disease that lies in the movement of uterus (from the Greek ὕστέρα *hystera* "uterus"). Hippocrates's theory was that since a woman's body is cold and wet compared to a man's body which is warm and dry, the uterus is prone to illness, especially if deprived from sex. He saw sex as the cleansing of the body so that being overemotional was due to sex deprivation.^[36] Plato described the phenomenon as "The animal within them is desirous of procreating children, and when remaining unfruitful...gets discontented and angry, and wandering in every direction through the body...drives them to extremity, causing all varieties of disease."^[37] Galen regarded hysteria as the result of a 'blocked menstrual flow and sexual abstinence'. He also believed that hysteria in men could be caused by 'retained sperm'. Galen's views persisted among medical practitioners of Christian post-Roman Britain.^[31]

Middle Ages

- The Trotula – a group of three texts from the 12th century—discusses women's diseases and disorders as understood during this time period, including hysteria. Trota of Salerno, a female medical practitioner from 12th-century Italy, is an authoritative figure behind one of the texts of the Trotula. (Authoritative in that it is her treatments and theories that are presented in the text). Some people believe Trota's teachings resonated with those of Hippocrates.^[36]

Renaissance

- The uterus was still the explanation of hysteria, the concept of women being inferior to men was still present, and hysteria was still the symbol for femininity.^[36]

Modern age

- Thomas Willis (17th century) introduces a new concept of hysteria. Thomas Willis believed that the causes of hysteria was not linked to the uterus of the female, but to the brain and nervous system.^[36]
- Hysteria was consequence of social conflicts during the Salem witch trials.^[36]
- Witchcraft and sorcery was later considered absurd during the Age of Enlightenment in the late 17th century and 18th century. Hysteria starts to form in a more scientific way, especially neurologically. New ideas formed during this time and one of them was that if hysteria is connected to the brain, men could possess it too, not just women.^[36]
- Franz Mesmer (18th century) treated patients with hysteria with his method called mesmerism, or animal magnetism.^[36]
- Jean-Martin Charcot (19th century) studied effects of hypnosis in hysteria. Charcot states that hysteria is a neurological disorder and that it is actually very common in men.^[36]

Contemporary age

- Sigmund Freud's work with Josef Breuer, *Studies on Hysteria*, contributes to a psychoanalytic theory of hysteria.
- Freud believed that hysteria was caused by a lack of libidinal evolution.^[36]

See also

- Acting out

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